

Is this telehealth company legit?

Five checks you can run yourself. Most of them take under two minutes.

FROM A PHYSICIAN ASSISTANT WHO'S BEEN ON A GLP-1 HERSELF

What's in here

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If you only have two minutes, go to the ten questions.

THEY ALL LOOK LEGITIMATE. THAT'S THE DESIGN.

Every one of these sites has a clean logo, a photo of a smiling clinician, and the word “licensed” somewhere near the top. That's the easy part to build. It tells you nothing.

Here's the number that reframed this for me: the National Association of Boards of Pharmacy has identified **more than 40,000 websites** that don't meet pharmacy practice standards or applicable law, and says **nearly 95% of websites offering prescription-only drugs online operate illegally.**

That is not a reason to be scared of telehealth. Real, careful telehealth companies exist, and they are a genuine door in for people whose insurance said no. It's a reason to stop judging by the website and start checking five specific things.

You don't need to be medical to run these. You need about ten minutes and two free websites.

CHECK 1 — A CLINICIAN LICENSED IN YOUR STATE

What you're looking for: the clinician's full name, and the state they're licensed in.

Why your state specifically: medicine is legally treated as happening where *you* are, not where the company is. So the person prescribing has to be licensed where you're sitting.

How to check, under two minutes: go to [docinfo.org](https://www.docinfo.org) and type the name. It returns license status, states, and disciplinary history. That covers MDs and DOs; for a PA or NP, use your state board's license lookup instead.

The honest limit: you usually can't run this until you've been assigned a clinician. Run it then. It is still worth doing after you've signed up.

CHECK 2 — THEY WILL NAME THE PHARMACY

This is the one most people skip, and it's the fastest. The app is not what's regulated — **the pharmacy is**. Most telehealth companies use a third-party pharmacy, and a company that won't tell you which one has told you something.

How to check, under two minutes: go to [safe.pharmacy](https://www.safe.pharmacy), run by the pharmacy boards. Use "Verify Before You Buy" and paste the pharmacy's website. You get back: verified, **Not Recommended**, or not reviewed.

A free tell: a web address ending in **.pharmacy** instead of .com means it was accredited — control of that ending is gatekept, so it can't be faked.

A second free tool: many telehealth sites show a **LegitScript Certified** badge. That one is checkable — go to [legitscript.com](https://www.legitscript.com), use the website-certification lookup, and type the company's web address. A badge is a picture; the lookup is the proof.

And one question almost nobody thinks to ask:

"Can my prescription be moved to a different pharmacy without telling me?" Some companies reserve exactly that right in their terms. It isn't illegal and it isn't rare — but "they named a pharmacy once" is not the same as "that pharmacy fills every refill."

CHECK 3 — IF IT'S COMPOUNDED, WHY FOR YOU?

This changed recently, and a lot of what's floating around is out of date. FDA determined the tirzepatide shortage resolved in December 2024 and the semaglutide shortage in February 2025. When a shortage ends, making a copy of the approved drug is no longer automatically allowed.

Now, compounding a copy is lawful only when a prescriber **determines and documents** that the compounded version has a change that makes a significant difference **for you specifically** — or the pharmacy fills very few of them.

So the question isn't "is compounded bad." It's: is it compounded, and if so, what's the documented reason it's right for me? A company that has updated its language will answer that cleanly. FDA's own consumer guidance is blunt: compounded drugs should only be used when an FDA-approved drug can't meet your needs.

A stated red flag: any company claiming a compounded drug "is the same as" an FDA-approved one. By definition it hasn't been reviewed by FDA for safety, effectiveness or quality.

CHECK 4 — SOMEONE IS STILL THERE AFTER THEY HAVE YOUR MONEY

Let me clear something up first, because a lot of vetting lists get this wrong.

Messaging a care team instead of one specific person is normal. In most of these programs — and in plenty of in-person practices — you message a team, and whoever is on that day answers you. You don't get to pick. **That is not a red flag.**
That's how it works.

What you're actually looking for

- **Someone credentialed is reachable after the medication arrives.** A licensed clinician, an actual response, in a reasonable time — not just a chatbot and a help-center article.
- **Clinical leadership has a name.** Somewhere on the site there should be a real person with real credentials — a medical director or chief medical officer you could look up. A company where *nobody* is named anywhere is different from one where you message a team led by someone named.
- **A plan across months, not one visit.** Dose changes and side effects handled over time, with a described process — not a single consult and goodbye.

Two of FDA's own red flags live here. A company is more concerning if it doesn't require screening and a prescription by a licensed clinician before providing medicine, or doesn't have a licensed clinician available to answer questions after you receive it.

My own read, as a PA: this is the check I'd weight heaviest. The first month is the easy month. What you need is for *someone qualified* to answer you in month three when something feels off — and it honestly does not have to be the same person every time.

CHECK 5 — THE PRICING IS TRANSPARENT. NOT CHEAP. TRANSPARENT.

Cheap and clear are different things, and clear is the one that protects you.

What you're looking for: the price, any recurring charges, and how to cancel — **disclosed before you're charged**. Not in fine print you find afterward.

This is not a small or theoretical thing. There's a federal law (ROSCA, 2010) that requires clear disclosure of terms, real consent, and a simple way to cancel before a company can bill you on a recurring basis. The FTC has brought cases against telehealth companies under it over subscription billing — including Hims & Hers and Cerebral.

The one that gets people: the intro rate is not the forever rate.

Nobody is exactly hiding this. It's just in smaller type. A plan can have one number for the first stretch and a different number after — and the second one is the one you actually live with.

If you can't tell what month three costs before you pay for month one, that is your answer.

Two things worth knowing. Many programs take **HSA or FSA** — that's real money most people forget they have. And “deep discounts or prices that seem too good to be true” is on FDA's own red-flag list. In this category, the cheapest option is a warning, not a win.

WALK AWAY IF

- They don't require a screening and a prescription from a licensed clinician.
- Nobody is reachable after the medication arrives.
- They say a compounded drug “is the same as” the FDA-approved one.
- They only take cryptocurrency or peer-to-peer payment apps.
- It ships from outside the US. **No Canadian pharmacy is licensed in the US**, and none that ships here is accredited, whatever the site says.
- It arrives warm, damaged, unlabeled, or with no instructions — these need cold chain.
- The label has spelling errors, or a pharmacy name that doesn't check out.
- It's sold “for research purposes” or “not for human consumption” but arrives with human dosing instructions.
- You can't find out what you'll be charged next month.

The questions. Screenshot this page.

Take them into the conversation. You are allowed to ask all ten — a good program won't mind a single one.

1. Is this price the intro rate, or the standard rate?
2. **What does my monthly cost become after the intro period ends?**
3. Does the price change when my dose changes?
4. Is the provider visit included, or billed separately?
5. Are there membership fees, setup fees, or anything added at checkout?
6. Does it ship with syringes and alcohol pads?
7. Is shipping included?
8. Do you accept HSA or FSA?
9. Is this compounded or brand name — and if compounded, why is that right for me specifically?
10. How do I cancel, and what happens if I need to stop, or it isn't working?

Number two is the one almost nobody asks. Ask it first.

WHERE TO GO FROM HERE

You got a no from insurance. That's not the end of the road — but it does mean you're the one doing the vetting now. So do it properly, once, and then stop worrying about it.

If you only do one thing: ask what month three costs, and make them name the pharmacy.

Got a denial letter and stuck on that part? I have a separate free guide on the three kinds of no and which ones are worth fighting — same place as this one.

Come find me: Instagram @diary_of_the_most_okayest_mom — I answer questions there, and the side-effect stuff nobody warns you about is most of what I post.

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